

NURSING DOCUMENTATION

Name:	Job Number:	Rating: 		
Unit:	Job Title:			
Contract Date:				
Evaluation Key: M- Met NM- Not Met NA- Not Applicable		Method of Evaluation: Knowledge: Exam(Written/Oral) Skills: Demonstration/Discussion Attitude: Observation		
Rating Scale: Met: 90% - 100% Not Met: 89% & below and remedial once NA-(Not applicable) – entries to be deducted from the total score				
COMPETENCIES		EVALUATOR ASSESSMENT		
		M (1)	NM (0)	NA
I.	KNOWLEDGE			
1.	Discusses what Nursing Documentation (OASIS PROGRAM) is.			
2.	Discusses the purpose of Nursing Documentation.			
3.	Discusses the proper and acceptable way of written/electronic documentation.			
4.	Discusses the legal implication and importance of proper documentation.			
II.	SKILLS			
1.	Documentation is done in narrative form.			
2.	Nurse's notes are started with complete assessment that is completed at the beginning of the shift.			
3.	Date and time of respective shift is entered in the column or space provided.			
4.	Nurse's notes are written accurately, pertinent, valid, timely and informative.			
5.	All entries are written concise and legible.			
6.	Chart potentially serious condition and/or observation, to which findings were reported, actions taken and ordered and patient's response to care and treatment.			
7.	English language is used in the medical records of patient that can be written and understood by all staffs.			
8.	Nurse's notes must be written in each succeeding lines. Draw a horizontal line to fill up a partially blank line. Blank/ space is not acceptable.			
9.	Date and time of every entry is written in the left margin of the nurse's notes and nurse's signature must be written on the right side. No nurse's notes or any entry will be left unsigned, no date and no time.			
10.	Mistaken Entry (ME) with the initial of the nurse will be written on top of the mistaken word/words after drawing a straight line on it.			
11.	Use of erasures, ink eradication, or use of occlusive materials is prohibited when making corrections.			
12.	Objective statements are used instead of judgmental interpretations and make sure quotation marks are used on patient's verbal complaints.			
13.	Use standard abbreviations, symbols, and official terms to avoid errors caused by misinterpretations.			
14.	Chart what had actually done and observed.			
15.	"Late Entry" is added/ written on the first available line, the time the event occurred when information/ entry was missed to chart earlier.			
16.	Documented client's refusal to medications and treatment. Encircled the time the medication is to be given on the medication sheet and enter the reasons in the nurse's notes.			
17.	Double charting was not observed but appeared/ written in the nurse's notes when there is an alteration from the normal.			
18.	Information like the time when the client/patient left and returned to the unit for some procedures or appointment to other specialty and the mode of transportation, medications including dosage and route of administration, whether pain is relieved for its purpose, presence of side effects is charted immediately after it was given			

COMPETENCIES		EVALUATOR ASSESSMENT		
		M (1)	NM (0)	NA
19.	Write only what has been done and observed.			
III.	ATTITUDE			
1.	Accept constrictive criticism as learning opportunity.			
2.	To be self-aware of limitation in knowledge and skills.			
3.	Ability to organize and facilitate a Reflection session in and on practice after a Nursing Documentation learning opportunity occurs and activates findings to improve future practice and response.			
	Raw Score			

Formula: $\frac{\text{Raw Score} \times 100\%}{\text{Total Score}} =$ % Rating

NEEDS REMEDIAL: YES NO
REMEDIAL DATE:

Evaluators Comments/Recommendations:

Evaluated By:

Evaluator's Name/Signature/Job Number

Staff Nurse Comments:

Conformed By:

Staff Name/Signature

Date